This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Podophyllotoxin or Imiquimod for the treatment of Genital Warts

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Genital Warts

Overview

- **Genital warts (caused by HPV types 6 and 11):** The most common viral sexually transmitted infection (STI). Non-oncogenic HPV types. Warts can be treated but virus may persist; recurrence is common.
- **Natural history:** Lesions may regress spontaneously, persist indefinitely, or recur after treatment.

Assessment

- **Clinical diagnosis:** Visual examination of visible warts on anogenital area (external genitalia, perianal region). Appearance ranges from smooth to keratinized, single to multiple lesions.
- **Sexual history and STI screening:** Number of sexual partners, barrier contraception use, concurrent STIs, immunocompromised status.
- **Cervical screening status (women):** Ensure up to date with cervical cytology as appropriate for age and HPV history.

- **Wart characteristics:** Number, size, location (external vs internal), keratinization, and patient preference for treatment modality.

Management

- **Patient-applied treatments:** Imiquimod 5% cream (immune modifier) or podophyllotoxin 0.5% solution / 0.15% cream (cytotoxic agent) for external warts.
- **Treatment choice:** Based on wart location, number, keratinization, size, and patient preference. Podophyllotoxin for small external warts; imiquimod for larger or keratinized warts.
- **Clinic-applied treatments:** Cryotherapy or trichloroacetic acid (TCA) available for alternative approaches.

Red Flags / Specialist Referral

- **Pregnancy:** Avoid treatment; defer until after pregnancy. Specialist referral if internal warts or complications.
- **Immunocompromised patients:** Higher risk of extensive disease, poor response to treatment, and recurrence. Monitor closely; may require specialist input.
- **Suspicious lesions:** Atypical appearance, bleeding, ulceration, or concern for malignancy. Biopsy to exclude squamous cell carcinoma or other pathology.
- **Internal warts:** Urethral, vaginal, cervical, or rectal warts require specialist assessment and treatment.

Patient Group Direction

for the supply/administration of

Podophyllotoxin 0.5% solution / 0.15% cream (Warticon/Condyline)

for the treatment of

Genital Warts

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working

	under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of small external genital warts. Patient-applied cytotoxic agent suitable for visible, non-keratinized external warts.
Inclusion criteria	Adults aged 18 years and over Visible external genital or perianal warts (not internal: urethral, vaginal, cervical, rectal) Small warts suitable for patient application: total treatment area up to and including 4 cm2 (the SmPC limit for unsupervised use) Able to identify warts and apply treatment to warts only (not healthy skin) Able to give informed consent
Exclusion criteria	Hypersensitivity to podophyllotoxin Pregnancy Breastfeeding Internal warts (urethral, vaginal, cervical, rectal) Open wounds or broken skin at application site Treatment area more than 4 cm2 (needs healthcare professional

	supervision; refer)
Cautions	Teratogenic: strict contraception required if any risk of pregnancy Avoid contact with healthy skin: apply petroleum jelly to surrounding area as protective barrier before treatment Do not exceed a treatment area of 4 cm2 for unsupervised application Solution: flammable; keep away from heat and ignition sources Wash hands immediately after application Avoid sexual contact while treatment applied (at least until washed off)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Podophyllotoxin 0.5% cutaneous solution (Warticon 3mL, Condyline 3.5mL) OR Podophyllotoxin 0.15% cream (Warticon 5g)
Legal category	POM
Storage	Below 25°C. Keep away from heat and light sources. Keep in original container.
Route/method of administration	Topical application directly onto warts (solution or cream)
Dose and frequency	Apply thin layer twice daily for 3 consecutive days (e.g., Monday-Wednesday morning and evening) Follow by 4 days rest (no treatment Thursday-Sunday) Repeat cycle: continue for up to 4 cycles (4 weeks total treatment, the licensed maximum) Review progress after 2 cycles; if warts persist, repeat for additional 2 cycles (a second pack may be supplied at this review only, and only where the first pack has been used)
Quantity to be administered and/or supplied	One pack per course: 1 bottle of 0.5% solution (Warticon 3mL or Condyline 3.5mL) OR 1 tube of 0.15% cream (Warticon 5g). One pack covers the licensed course of up to 4 weekly cycles. A second pack may be supplied only at the review after 2 cycles, where warts persist and the first pack has been used; no more than 2 packs per course.
Maximum or minimum treatment period	Up to 4 treatment cycles (each 3 days on, 4 days off); 4 weeks in total, the licensed maximum.

Adverse effects	Very common: local irritation, erythema, erosion, scabbing, pain at application site Common: burning, bleeding, edema, pruritus at wart site Rare (if significant absorption): systemic toxicity including myalgia, arrhythmias, neuropathy Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form and route, and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the podophyllotoxin product.
Follow-up advice to be given to patient or carer	Wash your hands after application of cream or solution (unless treating your hands). Apply treatment only to warts, not to surrounding healthy skin. Use Vaseline or petroleum jelly around warts to protect healthy skin. For podophyllotoxin: apply twice daily for 3 consecutive days (e.g. Mon-Wed morning and evening), then rest for 4 days (Thu-Sun). Repeat for up to 4 cycles. One pack normally lasts the whole course;

	a second pack is supplied only at your review after 2 cycles if the warts persist. Avoid all sexual contact while cream or solution is on your skin. Avoid sexual contact while the treatment is on the skin. Use condoms consistently with partners at other times, even if warts are treated, to reduce transmission risk. Discuss diagnosis and transmission risk with sexual partner(s); they may need screening and/or treatment. Complete the full treatment course even if warts shrink or appear to clear. Seek medical advice if warts worsen, spread significantly, or do not improve after completing treatment cycles. Report any severe local reaction, excessive pain, bleeding, signs of infection, or systemic symptoms (fever, severe headache) to your healthcare provider. Attend follow-up appointment to assess treatment response and plan any further management. HPV vaccination should be discussed, as it can help prevent other HPV types and recurrence. Pregnant women should inform their healthcare provider; treatment will be deferred until after pregnancy. Regular sexual health screening is recommended.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Genital warts
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which			

this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**

001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Imiquimod 5% cream (Aldara)

for the treatment of

Genital Warts

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products

	• All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of external genital and perianal warts, particularly larger or keratinized lesions. Immune response modifier activating local and systemic immunity.
Inclusion criteria	Adults aged 18 years and over External genital or perianal warts (not internal: urethral, vaginal, cervical, rectal) Able to identify warts and apply treatment correctly Able to give informed consent
Exclusion criteria	Hypersensitivity to imiquimod or any component of cream Internal warts (urethral, vaginal, cervical, rectal) Open wounds or broken skin at application site Pregnancy Breastfeeding
Cautions	Uncircumcised males: risk of phimosis (foreskin swelling). Advise careful application; if phimosis develops, cease treatment and refer to GP. Autoimmune conditions: may exacerbate. Monitor closely. Severe inflammatory reaction: temporarily cease treatment, consider rest period, or dose reduction Avoid application to large skin areas or non-wart skin: use petroleum jelly as barrier on surrounding healthy skin Wash hands thoroughly after application (unless treating hands) Avoid sexual contact while cream on skin (cream can transfer to partner)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Imiquimod 5% cream in sachets
Legal category	POM

Storage	Below 25°C. Protect from light and moisture. Keep in original container.
Route/method of administration	Topical application to external warts
Dose and frequency	Apply thin layer 3 times per week at bedtime (e.g., Monday, Wednesday, Friday) Wash off after 6-10 hours (e.g., next morning with soap and water) Continue treatment for up to 16 weeks Review progress at 8 weeks; if complete clearance, stop treatment
Quantity to be administered and/or supplied	12 sachets per dispensing (sufficient for 4 weeks at 3x/week dosing)
Maximum or minimum treatment period	Up to 16 weeks (continuous if tolerating well); review at 8 weeks for response.
Adverse effects	Very common (>10%): local erythema, erosion, excoriation, oedema, scabbing at application site Common (1-10%): local pain, burning, induration, itching, ulceration; systemic: headache, flu-like symptoms, myalgia, fatigue Uncommon (0.1-1%): fever, elevated liver enzymes, lymphadenopathy Rare (<0.1%): systemic immune-mediated events, SLE-like syndrome Yellow card report suspected adverse reactions via https://yellowcard.mhra.gov.uk

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form and route, and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should

	also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with imiquimod cream.
Follow-up advice to be given to patient or carer	Wash your hands after application of cream or solution (unless treating your hands). Apply treatment only to warts, not to surrounding healthy skin. Use Vaseline or petroleum jelly around warts to protect healthy skin. For imiquimod: apply 3 times per week at bedtime (e.g., Mon/Wed/Fri), and wash off the next morning after 6-10 hours. Continue for up to 16 weeks. Avoid all sexual contact while cream or solution is on your skin. Wash the cream off the skin before sexual activity. Imiquimod can weaken condoms and diaphragms, so they cannot be relied on while cream is on the skin; use condoms consistently at other times to reduce transmission. Discuss diagnosis and transmission risk with sexual partner(s); they may need screening and/or treatment. Complete the full treatment course even if warts shrink or appear to clear. Seek medical advice if warts worsen, spread significantly, or do not improve after completing treatment cycles. Report any severe local reaction, excessive pain, bleeding, signs of infection, or systemic symptoms (fever, severe headache) to your healthcare provider. Attend follow-up appointment to assess treatment response and plan any further management. HPV vaccination should be discussed, as it can help prevent other HPV types and recurrence. Pregnant women should inform their healthcare provider; treatment will be deferred until after pregnancy. Regular sexual health screening is recommended.

Key references

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https://www.nice.org.uk/guidance/mg2

- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Genital warts
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Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 25: podophyllotoxin is now one pack per course (one 3mL Warticon or 3.5mL Condyline bottle, or one 5g Warticon tube), with a second pack only at the review after 2 cycles where warts persist; the quantity row, the dose row, the medicine row and the follow-up advice all say so, and the 15mL bottle that is not marketed is no longer named.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document

above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover corrected to the supply of podophyllotoxin or imiquimod.

Podophyllotoxin treatment period corrected to up to 4 cycles (4 weeks, the licensed maximum), matching the dose row; it read 4 to 5 cycles.

Written information and follow-up rows in each arm now refer only to that arm's product.

Records row template text separated into date, dose, form, route and quantity, and the adverse reaction bullet no longer runs into 'supplied via PGD'.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

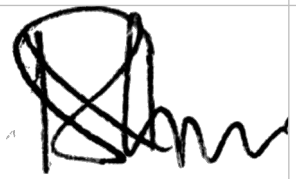
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Podophyllotoxin: the Warticon SmPC limits unsupervised application to an area of 4 cm2 and treatment to 4 weekly cycles. The previous version allowed 10 cm2 and up to 5 cycles, and quoted a 50-wart figure with no source. Corrected throughout.

Imiquimod: the Aldara SmPC warns that the cream weakens condoms and diaphragms and must be washed off before sex. The follow-up advice told patients to rely on condoms. Corrected.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.